
AFTER THE DEMO • FOR YOUR REVIEW

What you saw, and what we'd recommend.

A short, honest record of the demo — what it did on your own files, what it costs your staff in real terms, what happens when it doesn't work perfectly, and the one small thing we'd actually have you do first.

PREPARED FOR — TYPE B PRACTICE

Practices with an EHR, but a workflow stitched from disconnected tools.

You have an EHR. But billing, video, dictation and the queue don't talk to each other.

Written specifically for a practice in your situation.
The first step we recommend is the same for every practice — and it is deliberately the smallest one.

READ THIS
BEFORE YOU DECIDE.
IT IS DELIBERATELY SHORT.

You already run an EHR — so this was never about replacing the system you trust as your record. In the demo, we showed your legacy archive becoming structured data, exportable straight into the EHR you already run. Your system of record did not change, and would not.

— Your archive, structured — without touching your EHR

We took your own legacy files and extracted them into structured records, exportable into the EHR you already run. Your system of record was not altered.

— The questions your EHR can't answer

Most EHRs store data well and analyse it poorly. We asked the kind of cross-practice question your current system has no screen for, and got an answer with the source attached.

— Nothing about your record system disturbed

Export, not replacement. The archive flows into what you already run; nothing is taken away from a tool you already paid for.

The honest answer to the question you are really asking — “my reception is already drowning, what does this add to her day?”

<i>Per document</i>	30–60 seconds. The system extracts; your staff reviews what it found, corrects anything wrong, and approves. They know your handwriting; the system handles most of it; they catch the rest.
<i>Week one</i>	Scanning happens into the same folder your reception already scans into. The only new habit is the validation queue — a list of documents waiting for a 30-second check. That is the entire workflow change.
<i>Month three</i>	The queue is a background habit, not an event. The structured archive is large enough that the search and analytics start answering real questions. Nothing about the consult has changed.
<i>What does not change</i>	How you consult. What you write. Your existing system, if you have one. We are a layer that adds capability, not a system that takes work away from one that already works.

Demos show the happy path. This is the part that matters more: what the system does when a scan is bad, the handwriting is unreadable, or the extraction is unsure. We would rather you read this now than discover it later.

Low-confidence extraction

Every extracted field carries a confidence score. Low-confidence fields are flagged for the validator, not silently accepted. The human sees exactly what the system was unsure about.

An unreadable document

If a scan is too poor to extract, it does not get guessed at. It is held in the queue, flagged, and a person decides — rescan, enter by hand, or set aside. The system never invents data to fill a gap.

The wrong-patient safeguard

Nothing enters a patient's record until a person confirms it is the right patient. The match is shown; the human says yes or no. Wrong-patient errors are caught at that gate — before they happen, not discovered months later.

Every action is logged

Every approval, correction and reversal is recorded — who did it, when. If a medical aid audits the practice, or you need to show the basis of a record, the history is one query away.

04 WHY A SOUTH AFRICAN PRACTICE CHOOSES THIS

BUILT HERE, NOT BOLTED ON

Medical-aid schemes, HPCSA prescription rules, NAPPI codes, the SA edition of ICD-10, local payments — native to the system, not adapted to it after the fact.

IT WORKS WITH WHAT YOU ALREADY HAVE

Export and ingest via FHIR, CSV or API. You do not rip out a system that works to get the parts you don't have.

DIGITISATION AS A SUBSCRIPTION, NOT A PROJECT

The reason practices stay on paper is the six-figure project quote nobody signs. A predictable monthly page allowance instead — payable, cancellable, no capital sign-off.

EACH PRACTICE'S DATA IS ISOLATED

Multi-tenant by design. What happens at one practice never crosses to another.

The same first step regardless of how your practice is set up today — because every practice, paper or EHR or fully wired, has the same one gap that digitisation closes: a back room of legacy files that is dead weight until it is structured data.

Digitisation only. Nothing else, to start.

Not a platform. Not analytics. Not a bundle. The single thing we would put in your practice first is the digitisation of your legacy archive — because it is the one thing every practice needs, it is complete and useful entirely on its own, and it changes nothing about how you already work.

A real, complete product — not a foot in the door

Digitisation is not a teaser for something else. A practice that only ever digitises, and never buys another module, has still solved a real problem permanently. We mean that; the rest of this document is written so you can hold us to it.

Priced as a subscription, not a project

A fixed monthly page allowance, payable and cancellable, no six-figure project quote. We work through the archive at that pace. It takes time — and the predictability is the point, for you and for us.

Everything else this company sells is downstream of this one step — and it is downstream for a concrete reason, not a commercial one. See the next section.

This is the single most important thing in this document, so we are going to state it as the fact it is, not as a sales line. Analytics cannot run on a filing cabinet. The platform's value does not compound without structured history. Every capability beyond digitisation — the view of your practice, the cohorts, the search, anything further — needs the structured data that digitisation, and only digitisation, produces. That is not a reason to buy more. It is the reason the order is fixed: the archive becomes a data asset first, and whatever you choose to do with that asset comes later, on your timing. A vendor that sold you the downstream modules before the data existed would be selling you something that cannot work yet.

This section is not what you are being asked to decide now. It is here so you can see where digitising your archive leads — and the key point is that each step below is possible *only because* digitisation produced structured data first. It is a later conversation, and your EHR stays the system of record throughout.

Once the archive is structured — the analytics view

Ingested from your existing EHR, the analytics layer answers the cross-practice questions your current system stores the data for but cannot surface. This is the most natural next step, and it depends entirely on the structured data digitisation is already producing — not on changing your EHR.

If the tool-stitching gets worse — consolidation

If the evening spent stitching billing, notes, video and the queue together becomes the thing you most want to fix, the platform can end that patchwork — without becoming your system of record. That is a real decision for later, not part of this. We are not telling you your EHR was the wrong choice; we are noting where the workflow pain leads if you choose to address it.

The order is not negotiable: digitisation first, because the analytics and the consolidation both feed on the structured data it produces, and because it is the one step that touches your EHR least. We would rather prove the data asset against your own documents than ask you to rethink your workflow on day one.

The same discipline we apply to the product, applied to this document.
These are real and they are not in scope of what you would be buying.

FINAL PRICING

Set per practice on doctor count, page volume and integration scope. The pilot price is fixed and stated; the full quote is a conversation, not a number on a leave-behind.

CLINICAL DECISION SUPPORT

Our clinical-AI work is an invite-only beta, not a commercial product, with no medical-device approval. It is a design-partner conversation if you raise it — it is deliberately not part of what this document offers you.

ROADMAP ITEMS

Medical-aid claims auto-submission, multi-site management and native EHR integrations are planned, not promised here. If it is not in this document, treat it as not yet real.

Not the catalogue. Not your type's eventual configuration. One small, bounded, cancellable first step — the same one we'd recommend to any practice, because it is the one that earns everything after it.

THE SMALLEST FIRST STEP

One month. Your files. Digitisation only.

Not the whole archive. Not a year's commitment. Not a bundle.

Digitisation only: one month, 1,500 pages of your choosing — ideally the messiest ones, so you are testing the hard case. Your staff validates as they would in production; the output lands in whatever system you already run, or in ours if you have none. At the end of the month you have a real, structured slice of your own archive and an honest sense of the day-to-day. No setup fee, no lock-in, cancel any time. Everything else is a later conversation that this step earns — or doesn't.

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